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Learned helplessness in humans: A contemporary theoretical reappraisal

Clement F.A. Andrieu [a,*], Isabelle Milhabet [b]

[a] Univ Gustave Eiffel, Université Paris Cité, LaPEA, Versailles, F-78000, France
[b] Université Côte D'Azur, LAPCOS, France

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ABSTRACT

Learned helplessness is a foundational concept in psychology, originally developed to explain the effects of repeated exposure to uncontrollable aversive events on behavior and adaptation. Over the past sixty years, the theory has evolved through successive reformulations, accumulating both conceptual depth and ambiguity. In this critical review, we revisit the historical development of learned helplessness, outlining its core mechanisms while highlighting the tensions that have emerged across theoretical traditions. Particular attention is given to the past three decades, during which learned helplessness has been increasingly reinterpreted, expanded, and applied across diverse research areas and real-world contexts. It is timely to examine how these developments have transformed the scope and meaning of the concept, sometimes in ways that depart from its original foundations. While analyzing this trajectory, we identify key points of convergence and divergence in the field and consider how learned helplessness continues to inform contemporary models of motivation, resilience, and adaptation. Finally, we propose an integrative framework to clarify its current relevance and offer directions for future research, grounded in coherent theoretical principles and informed by empirical insights.

Learned helplessness (LH) refers to a state of motivational, cognitive, and emotional impairment that results from learning that there is no contingency between actions and outcomes. This learning generally occurs in the context of repeated failure to exert control over negative or aversive outcomes. Since its emergence in the late 1960s, LH has had a considerable influence on behavioral science, shaping research on stress, coping, depression, and perceived control. Its theoretical trajectory has evolved through different stages and reformulations. The latest of these, the informational reformulation of the 1990s, emphasized the role of cognitive processing breakdown under uncontrollability. In the decades since, no comprehensive reappraisal has been undertaken of the theory or of how it is studied in contemporary psychology. As illustrated in Fig. 1, LH is not only still cited and debated, but has seen a renewed and growing presence in recent scholarly literature. Still, questions persist about its mechanisms, its boundaries with adjacent psychological constructs, and its applicability across contexts.

Over time, LH has been diversely adopted, revised, overlooked, or reconsidered in light of shifting theoretical and empirical priorities. In this review, we examine LH from theoretical, empirical, and applied perspectives, tracing its evolution from its original formulation (1960s–1990s) to contemporary developments (1990s–present). Where relevant, we also consider perspectives from outside the cognitive tradition, including the experimental analysis of behavior. We conclude with a critical appraisal of the current state of research on LH and propose a conceptual framework to guide future investigations. Our aim is to clarify the enduring contributions of the construct, identify unresolved tensions, and explore its relevance within contemporary models of behavior and adaptation.

1. A brief review of the historical trajectory of learned helplessness (1960s–1990s)

1.1. The origins of the theory (1960s–1970s)

The LH theory was first formulated toward the end of the 1960s, based on experiments with dogs exposed to uncontrollable electric shocks (Maier & Seligman, 1976; Seligman & Maier, 1967). In these experiments, animals were restrained and received shocks they could not avoid, regardless of their behavior. Later, when placed in a different situation where escape was possible (typically a shuttle box with a low barrier), the dogs made no attempt to flee, displaying passivity and behavioral disengagement. This contrasted with dogs from other experimental groups, that had either not been exposed to shocks or had experienced shocks they could actively avoid (by pushing a panel with

* Corresponding author.
E-mail address: clement.andrieu@univ-eiffel.fr (C.F.A. Andrieu).

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their nose) and had learned to escape in the new setting. In this original behavioral formulation, LH was defined as a state resulting from repeated experiences of uncontrollability, referred to as repeated occurrences of action–outcome non-contingency (Overmier & Seligman, 1967). This state was reported to have three main consequences (Seligman, 1975): a motivational deficit (reduction in initiating behavior), a cognitive deficit (difficulty in learning new contingencies), and an emotional disturbance (increased anxiety or depressive-like symptoms).

The theory was extended to human subjects through adapted experimental paradigms involving noise, failure feedback, or insoluble tasks (Benson & Kennelly, 1976; Gatchel et al., 1977; Hiroto & Seligman, 1975; Jones et al., 1977; Klein et al., 1976). These studies replicated the key effects observed in animals, namely that non-contingency between behaviors and outcomes can be learned and that behavioral, cognitive and emotional deficits are the consequence. These results reinforced the relevance of LH as a model of human psychological functioning, particularly in relation to depression (Miller & Seligman, 1975; Seligman, 1975).

Early LH studies (both in animals and humans) were frequently criticized for their limited ecological validity (Costello, 1978). The reliance on laboratory paradigms raised concerns about the extent to which these findings could be generalized to real-world contexts. Moreover, the applicability of animal models to human behavior was also questioned. Finally, the original behavioral formulation failed to account for individual variability in responses to uncontrollability. This limitation prompted growing interest in potential moderating factors, such as prior exposure to control (the so-called "immunization effect"[1]; Jones et al., 1977), dispositional traits (Ames, 1978), or cognitive evaluations of different types of failure to control aversive events (e.g., attributions; Hanusa & Schulz, 1977).

1.2. The era of theoretical reformulations (1970s–1990s)

The first major shift came with the attributional reformulation (Abramson et al., 1978), which emphasized the role of individuals' causal explanations for their inability to control negative events. LH was considered more likely to emerge (i.e., to manifest stronger behavioral, emotional, and cognitive impairments) when individuals attributed repetitive failure to internal ("it's my fault"), stable ("it will always be this way"), and global ("it affects everything in my life") causes. This framework also aimed to explain why LH consequences could persist and be generalized across situations and over time. For instance, attributing repeated failure to internal, stable, and global causes (e.g., "I keep failing my driving test because I'm incompetent at everything I try") is more likely to lead to pervasive deficits than more specific, external attributions (e.g., "I failed this time because the evaluator was a jerk").

A related approach, the egotism explanation (Frankel & Snyder, 1978), focused on self-esteem. This view argued that when individuals perceived themselves as personally responsible for repeated failure, LH became tied to negative self-evaluations, compounding motivational and emotional decline.

These cognitive models marked a significant evolution of the LH theory. However, they also introduced new challenges. The focus on internal, stable, and global attributions proved too narrow to account for the diversity of explanatory factors across individuals and contexts (and in some cases, individuals do not engage in attributional reasoning at all; Gotay, 1985). Proponents of the theory were also divided between those who considered attributional style a stable dispositional trait (Alloy et al., 1984; Peterson et al., 1982) and those who saw it as a situation-dependent response (Cutrona et al., 1984), which generated inconsistencies in both theoretical framing and empirical results.

A third reformulation of the LH theory emphasized the role of informational and cognitive processing mechanisms (Kofta, 1993; Kofta & Sędek, 1989). Rather than focusing on how individuals explain negative events, this perspective explored how exposure to incoherent outcomes outside their control disrupts cognitive functioning itself. Such disruptions lead to informational overload and mental fatigue, resulting in reduced behavioral initiative, impaired performance on subsequent tasks, and emotional dysregulation (Kofta, 1993; Kofta & Sędek, 1989; Sędek et al., 1993; Sędek & Kofta, 1990). In this view, LH (and its generalization to other situations) emerges not only from stable beliefs but from a breakdown in cognitive processes, including attentional depletion, executive dysfunction, and diminished problem-solving capacity. This model broadened the conceptual scope of LH by showing that repeated action–outcome non-contingency can impair goal-directed behavior even in the absence of explicit attributional reasoning.

By the mid-1990s, LH had evolved into a multilayered construct integrating behavioral, cognitive, and informational components. These reformulations expanded the theory's scope, linking uncontrollability to

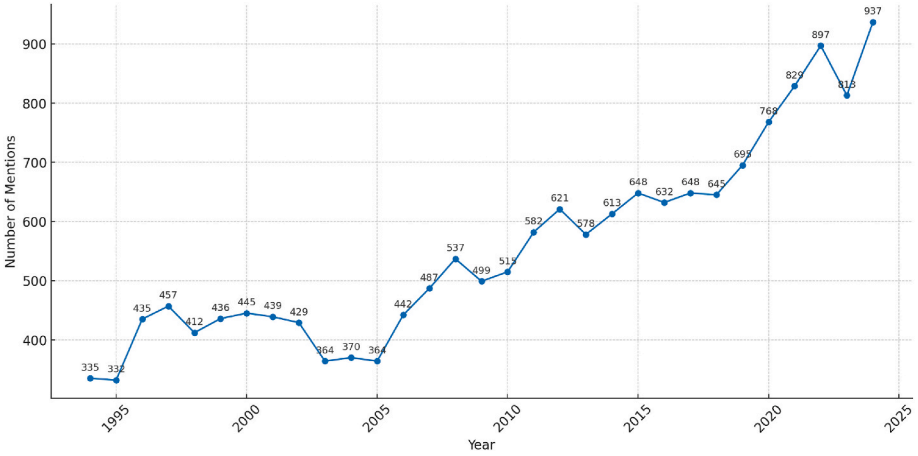


Fig. 1. Mentions of Learned Helplessness in Publications per Year Between 1994 and 2024

Note. This figure shows the annual number of publications indexed in Scopus that contain the term "learned helplessness" in any indexed field (e.g., title, abstract, keywords, references). The search was conducted in September 2025 using the advanced query ALL ("learned helplessness").

[1] The idea that prior exposure to controllable stress can buffer individuals against the negative effects of later uncontrollable stress, by reinforcing a sense of efficacy or control.

various psychological processes and extending its relevance across domains. However, they also left unresolved tensions, particularly regarding the coherence of the construct and the precise mechanisms by which uncontrollability leads to motivational, cognitive, and emotional impairment. Persistent questions remained about the generality of LH-related deficits and the role of moderating factors such as prior control and cognitive appraisals. Whether these challenges have been addressed in the decades since remains an open question.

1.3. What is "learned helplessness" exactly?

Rather than committing to one reformulation over another, we have adopted an integrative perspective aimed at synthesizing the core elements that define LH across the cognitive theoretical traditions developed above. The LH phenomenon can be broadly understood as composed of two major interrelated components: (1) the process by which LH develops (antecedents and mediating internal processes) and (2) the range of outcomes it produces.

The antecedents involve repeated exposure to non-contingency between actions and outcomes, typically in the context of aversive or negative events. Crucially, this exposure must be repetitive rather than isolated. The development of LH is then mediated by several internal processes. First, the individual must—consciously or unconsciously—perceive the lack of contingency between actions and outcomes. Second, this perception must be learned and integrated, meaning the individual internalizes that personal behavior no longer influences results. This is followed by the interpretation of this uncontrollability, conceptualized differently depending on the theoretical framework adopted. The attributional reformulation emphasizes causal interpretations, whereas the informational reformulation focuses on the mental effort to generate alternative behavioral hypotheses to regain control and the subsequent cognitive and motivational fatigue that arises when these fail.

The second key component of LH involves its observable consequences, which are typically manifest across three domains: (a) behavioral (e.g., passivity, disengagement), (b) emotional (e.g., increased negative affect, low self-esteem), and (c) cognitive (e.g., impaired learning, reduced problem-solving ability, performance decline).

As a rule of thumb, in order to properly identify LH, there must be evidence (ideally derived from rigorous methods such as control group comparisons) that the observed phenomenon includes (1) repeated exposure to non-contingency between actions and outcomes, (2) internal processing of this uncontrollability, and (3) observable behavioral, emotional, and cognitive consequences. The presence of these three components also led LH to be conceptualized, particularly in early experimental paradigms, as a form of "induced" or "experimental" depression, in the sense that it simulates affective and behavioral symptoms of depression through repeated exposure to uncontrollable failure (Miller & Seligman, 1975).

To illustrate the concept, consider an individual whose car repeatedly breaks down despite consistent maintenance efforts and professional repairs. Each attempt to resolve the issue is met with new or recurring problems (e.g., engine failure, battery issues), none of which the person can predict or control. Over time, the person not only stops trying to fix or to drive the car but also begins to internalize the breakdowns—believing to be inherently incompetent, to "ruin everything I touch," or to be fundamentally incapable of managing responsibilities. This perception spreads beyond driving, diminishing motivation in other areas of life (e.g., household chores, work, relationships).

This trajectory illustrates the three main aspects of LH: repeated non-contingency, internal processing of inefficacy, and cross-domain behavioral and emotional disengagement. Now, consider another driver facing similar breakdowns but making a different type of attribution. This driver attributes them to specific mechanical causes (e.g., poor repair work by the mechanic, old parts) and maintains confidence in the ability to resolve the issue. This driver remains behaviorally engaged (e.g., by seeking another mechanic, a different parts supplier, or by switching to another vehicle) and does not exhibit emotional or cognitive collapse. Here, the repeated setbacks do not lead to LH, as the experience of uncontrollability is neither internalized nor generalized. It reflects a type of situational helplessness associated with adaptive coping, rather than *learned* helplessness.

1.4. Is the non-contingent operation sufficient?

The proposed definitions of LH and the different reformulations reviewed above share one assumption: that non-contingency sets in motion internal mediating processes which in turn produce the observed deficits. For completeness, it is worth noting that one tradition, the experimental analysis of behavior, rejects this mediational move altogether, a perspective rarely represented in the largely cognitive LH literature.

Its starting point is that "uncontrollable shock" is one instance of a more general operation: the non-contingent presentation of a stimulus. When the stimulus is appetitive (e.g., food), the same operation (non-contingent reinforcement, or NCR) also reduces operant responding (Phillips et al., 2017). If both appetitive and aversive non-contingent operations produce comparable decrements, the active variable is unlikely to be the hedonic quality of the event; it is the non-contingency itself, with the affective tone foregrounded by cognitive accounts being incidental to the functional relation. More broadly, behavior is held to be governed by the prevailing schedule rather than by the nature of the scheduled events (Keenan & Toal, 1991; Morse & Kelleher, 2022). On this view, the search for mediating cognitions does not explain the deficit but merely redescribes it: since none of these states would arise in the absence of the non-contingent operation, positing them draws attention away from the functional relation that does the explanatory work (Baum, 2011; Keenan, 1999; Skinner, 1977).

The original LH paradigm is itself an escape task: the dependent measure is failure to escape, and the deficit is a failure to acquire the escape or avoidance response. The operant tradition analyzes such behavior directly in terms of negative reinforcement, avoidance, and escape, without recourse to internal mediators (Hineline, 1977; Iwata, 1987). Indeed, work on the conditions under which escape responding is acquired, maintained, or extinguished (Iwata et al., 1994) suggests that what LH theory attributes to a learned expectation of uncontrollability may be redescribed as the weakening of escape behavior under the prevailing aversive contingencies.

The objection cuts deep, as it targets the postulation of an internal process between contingency and behavior that is common to every LH reformulation discussed above. We take up its implications in the General Discussion.

2. How has learned helplessness evolved over the past three decades (late 1990s–Present)?

Since the late 1990s, no major reformulation of the LH theory has been proposed. Research has nonetheless continued to evolve in distinct ways, though each of the trajectories below retains the mediational assumption just questioned, whatever their other differences. Some approaches to LH have deepened (and been based upon) earlier reformulations, extending their application across new domains such as education and health. Others have pursued a more technologically driven path, employing biological and physiological methods to examine how repeated uncontrollability affects behavior and cognition, often without clear alignment with any specific reformulated model. Finally, others have returned to the original formulations of LH, seeking to reinterpret and refine them. These trajectories reflect the theory's enduring relevance, but also the diversity and occasional divergence of contemporary approaches.

2.1. Approaches based on past reformulations

2.1.1. Attributional reformulation

The attributional model has remained the most influential in clinical and educational contexts. It offered a framework for understanding vulnerability to depression, notably the hopelessness theory of depression (Abramson et al., 1989). This theory posited that individuals who tend to explain negative events with internal, stable, and global attributions are more likely to experience persistent passivity, emotional distress, and impaired cognitive engagement. This explanatory style has been widely studied as a cognitive factor of vulnerability to depression and suicidal ideation, particularly among adolescents and young adults.

The rise of positive psychology in the early 2000s gave further momentum to this line of work through the development of learned optimism (LO), a concept spearheaded by the original author of the LH theory (Seligman, 1990, 2002; Seligman et al., 1995). Rather than focusing exclusively on cognitive vulnerability, LO reframed the attributional response as a target for preventive and developmental intervention. In programs such as the Penn Resiliency Program (Gillham et al., 1990), cognitive-behavioral techniques are used to teach people how to reinterpret failure in terms that are less global (e.g., "I struggle in math, but I'm good at other subjects"), less stable (e.g., "This won't always be the case, I can improve with practice"), and less internal (e.g., "The task was unusually difficult for everyone.") to promote resilience.

Building on these foundations, several cognitive-behavioral interventions have targeted maladaptive attributional patterns. These interventions have shown moderate success in improving mood and enhancing perceived control among individuals with mild to moderate depressive symptoms (Haeffel et al., 2017; Henkel et al., 2002). However, their efficacy appears more limited in cases of severe depression, especially when it comes to reducing suicidal ideation (Cuijpers et al., 2013). In recent years, these attribution-focused interventions have also been somewhat eclipsed by more comprehensive cognitive-behavioral frameworks, such as acceptance and commitment therapies (Hayes & Hofmann, 2017; Twohig & Levin, 2017).

Interventions based on positive psychology have also been the subject of critical scrutiny. Systematic reviews have found their effects to be modest and highly dependent on implementation quality and contextual fit (Brunwasser et al., 2009). Critics have also noted an overemphasis on individual cognitive reframing, which tends to place the burden of adaptation solely on the individual while neglecting structural and systemic contributors to helplessness (Mumford et al., 2023; van Zyl et al., 2024). For example, students in an underfunded school may be coached to "believe in themselves" despite poor access to qualified teachers or learning materials—implicitly suggesting that motivation, rather than resource disparity, is the main problem. Moreover, it has been argued that LO reflects Western, individualistic values such as achievement orientation, control over destiny, and rational individualism. In collectivist contexts, where coping is often relational, its emphasis on personal agency may clash with local understandings of suffering and resilience (van Zyl et al., 2024). Additionally, the spread of optimism-based programs into corporate and self-help domains has raised concerns about the commodification of psychological science and the dilution of the original LH model's theoretical and ethical foundations.

2.1.2. Informational reformulation

Building on the informational reinterpretation of LH, recent studies have more precisely identified which cognitive functions are impaired under prolonged uncontrollability. Across various paradigms, LH has been associated with deficits in working memory, attentional flexibility, problem-solving efficiency, and broader executive functioning (Bukowski et al., 2015; 2019; 2024). Furthermore, some studies have pointed to disruptions in intentional binding (i.e., the perceived causal link between one's actions and their outcomes) suggesting that the sense of agency itself may be compromised under sustained action–outcome non-contingency (Soral et al., 2021). These findings support the idea that LH is not solely due to a biased interpretation but also reflects a breakdown in cognitive control systems.

2.2. Neurobiological approaches

Since the mid-1990s, new methodological tools—particularly from neuroscience—have reshaped how LH is studied. Moving beyond self-report and behavioral paradigms, researchers have begun to investigate the underlying neural mechanisms that sustain helpless states. While these approaches still study the effects of repeated uncontrollability, they are less concerned with attributional or informational reformulations and more focused on identifying biological and neurocognitive correlates of passivity, disengagement, and motivational shutdown.

One insight concerns the prefrontal–amygdala circuit, which plays a key role in regulating emotions and supporting goal-directed behavior. Under systematic uncontrollable stress, increased activity of catecholamines (such as dopamine and norepinephrine) disrupts the normal functioning of the prefrontal cortex (PFC), leading to difficulties in executive control, reduced attentional flexibility, and impaired planning (Arnsten, 2015; Datta & Arnsten, 2019). In turn, when the PFC is no longer able to regulate the amygdala effectively (via top-down regulation), emotional responses become stronger, and individuals are more likely to adopt passive or avoidant behaviors (Banks et al., 2007; Berboth & Morawetz, 2021).

Equally critical are the mechanisms outlined by Maier and Seligman (2016) linking uncontrollable stress to passivity. Their review shows that sustained aversive stimulation activates the dorsal raphe nucleus (DRN), a serotonergic brainstem structure that regulates behavioral withdrawal and anxiety. This reaction is an unlearned "biological default response" that emerges automatically in the absence of control. The ability to counteract it depends on whether control has been learned, which engages a circuit between the ventromedial prefrontal cortex (vmPFC) and the dorsomedial striatum, which detects controllability. Once control is detected, the vmPFC sends inhibitory signals to the DRN, suppressing its output and enabling active coping. This process underlies the "immunization effect," whereby prior experiences of control produce lasting resilience against later uncontrollable stress (Maier & Seligman, 2016). Animal studies further demonstrate that uncontrollable stress sensitizes DRN neurons, maintaining a bias toward passivity unless overridden by vmPFC-mediated inhibition. In contrast, animals with a history of controllable stress readily engage this inhibitory pathway under new stress, preventing helplessness and sustaining adaptive behavior (Amat et al., 2005; Maier & Watkins, 2005). This vmPFC–DRN mechanism thus adds a distinct layer to current LH models by identifying a specific neural circuit that detects and overcomes the default passive state.

These neuroscientific insights suggest an interesting conceptual shift: LH may not result from learned passivity per se, but rather from the failure to detect action–outcome contingency after repeated exposure to uncontrollability. This reframes resilience as the learned capacity to override passivity, aligning with, but subtly reorienting, traditional LH theories. Notably, treating passivity as an unlearned default rather than an acquired belief partly echoes the behavior-analytic counterpoint raised above, locating the explanatory weight in the organism's exposure to (un)controllability rather than in its interpretation of it. Still, most of these findings come from animal models using highly controlled aversive paradigms, which limits their generalizability to humans. Thus, while these insights deepen our understanding of the neural mechanisms underlying LH, they complement rather than replace psychological and contextual accounts.

2.3. Theoretical reinterpretations

While clinical and neurobiological approaches have dominated the

study of LH in recent decades, a smaller body of work has focused on revisiting, reinterpreting, and refining the theory's foundational assumptions. These conceptual contributions often aim to reassess the boundaries of the construct, or integrate LH within broader models of motivation, self-regulation, or stress adaptation (Brandstätter, 2022; Duda & Joormann, 2022; Scherer, 2022).

One proposition is the integration of LH into models of goal regulation and action potential (Boddez et al., 2022). Its proponents suggest that repeated exposure to non-contingent outcomes modifies how individuals evaluate their personal goals, often leading them to devalue those goals or disengage from future challenges altogether. They argue that, beyond perceiving outcomes as uncontrollable, individuals begin to internalize a diminished sense of action relevance, which in turn weakens their capacity to initiate new goal-directed behaviors. For instance, a student who repeatedly fails at difficult math problems may begin to disengage not only from math, but also from broader academic challenges perceived as similarly demanding or evaluative. The experience of uncontrollability becomes associated with a class of goals ("tasks that require intellectual effort") rather than with a single activity. According to this model, helplessness can spread through perceived goal similarity and thematic overlap, rather than solely through stable attributional styles. Although this motivational reinterpretation of LH is theoretically promising, empirical support for it remains limited and it calls for further investigation.

Another perspective is that under conditions of acute and inescapable stress, LH may serve as an adaptive disengagement strategy (Lifshin et al., 2020). In such contexts, withdrawal of effort and lowered expectations of control can operate as a regulatory mechanism that minimizes cognitive load and emotional distress when action appears futile. Lifshin et al. (2020) examined psychological responses during the COVID-19 pandemic and found that both high and low levels of perceived helplessness regarding the possibility of avoiding infection were associated with reduced anxiety relative to moderate levels—indicating a curvilinear relationship. This pattern suggests that extreme perceptions of helplessness may, in some cases, buffer against fear by reducing the attentional and motivational demands of sustained threat.

Although this defensive characteristic of LH remains rather underexplored, it contributes to a nuanced understanding of passivity. Nevertheless, the characterization of such responses as LH is questionable. In the absence of prior conditioning or generalization of repeated uncontrollability, these patterns may simply reflect situational acceptance of the absence of direct control or tactical withdrawal—strategic responses to perceived futility rather than actively learned beliefs about one's lack of action–outcome contingency.

Building on the perspective of LH as goal regulation (Boddez et al., 2022), Mikulincer and Lifshin (2022) propose an existential–motivational framework, integrating insights from attachment theory and terror management theory. They argue that LH spreads more widely when uncontrollability threatens goals that serve as existential "buffers" (e.g., attachment, cultural worldview validation, self-esteem), which protect individuals from anxiety associated with mortality, meaninglessness, and isolation. From this standpoint, LH may not solely reflect a breakdown of agency but can also emerge as a motivated response aimed at reducing existential anxiety. For example, the authors illustrate how failure to obtain recognition in one's career (a source of symbolic self-worth) may lead individuals to compensate by binding to attachment figures, and how compounded failure across multiple existentially salient domains may culminate in intensified LH and even suicidal ideation.

This framework complements the goal-based perspective (Boddez et al., 2022) by clarifying why failure tied to existentially central goals (e.g., the need for belonging) is more likely to elicit LH than failure in more peripheral or instrumental domains (e.g., cooking or driving). In many ways, however, it closely echoes the egotism explanation (Frankel & Snyder, 1978), which also conceptualized LH as a withdrawal aimed at protecting self-esteem in the face of repeated failure. The added value of this existential-motivational model lies primarily in broadening the range of psychological "buffers" involved but its core mechanism remains conceptually similar.

Together, these developments remodel LH as a dynamic, context-sensitive process. They introduce new candidate mediators such as goal appraisal, perceived action relevance, or the stability of existential buffers, that may influence how LH emerges and is generalized. However, empirical validation of these theoretical frameworks remains limited, underscoring the need for integrative research that connects these emerging models with foundational findings.

3. A living concept: contemporary applications across domains

Whether explicitly tied to one of the major theoretical reformulations or used more loosely as a conceptual reference, LH remains a popular heuristic for interpreting real-world phenomena in contemporary psychology. In what follows, we outline a few (though non-exhaustive) illustrations of research lines that have been based on the LH framework in the past 30 years. In some cases, this application is conceptually anchored; in others, it is more partial or metaphorical, exposing the concept to risks of definitional drift.

3.1. Health and well-being

Beyond psychopathology (addressed earlier in the review), LH has been applied to substance use research, namely in understanding relapse vulnerability and maladaptive coping (Sterling et al., 1996; Wang et al., 2017). It has been used to explain how repeated failure to maintain abstinence can undermine self-efficacy and foster passivity in future attempts to abstain (e.g., Hitzemann, 2000). LH has also been shown to influence how individuals respond to treatment. Those with "high levels" of LH tend to respond better to structured, directive interventions, while those with "lower levels" of LH benefit more from flexible, less directive approaches (Thornton et al., 2003).

In the context of chronic illness and pain, LH has served as a useful lens for understanding patients' disengagement and psychological distress, particularly when repeated medical interventions or therapeutic efforts fail to produce relief. For example, among individuals with fibromyalgia, "feelings of helplessness" have been shown to mediate the link between pain and depressive symptoms (Nicassio et al., 1999). More broadly, LH has been identified as an important predictor of pain intensity, disability, and depression in populations with chronic pain, highlighting its clinical relevance as a target for intervention (Samwel et al., 2006).

Applications of LH have also extended to occupational health, where it has been used to explain behavioral withdrawal and emotional exhaustion in high-demand, low-control work environments. When sustained effort fails to influence outcomes, employees may experience reduced motivation, drops in performance, and psychological fatigue—symptoms consistent with LH (Bianchi et al., 2015). These effects have been linked to broader indicators of workplace dysfunction and diminished well-being (Khamisa et al., 2013). However, such applications often use metaphorical extensions of the concept. LH is typically invoked and inferred from perceived lack of control, rather than the core processes of learning and interpreting non-contingency that define the phenomenon (e.g., Andrieu et al., 2024).

3.2. Performance-driven situations

LH has been widely applied in performance-oriented settings to explain motivational and emotional impairment following repeated failure. In education, for instance, students who perceive their efforts as ineffective in the face of persistent academic setbacks often exhibit symptoms that researchers associate with LH (e.g., reduced persistence, diminished self-efficacy, and avoidance of future challenges; Filippello

et al., 2020). The construct plays a role in motivation research, informing interventions aimed at strengthening students' sense of control and mastery.

Similar dynamics have been observed in sports psychology, where LH has been used to interpret declines in motivation after consistent underperformance. Athletes exposed to repeated setbacks despite sustained effort or preparation may disengage or limit their efforts, exhibiting patterns of passivity aligning with LH (Sankaran, 2018). The concept has been used to explain decreased confidence, withdrawal from competition, and reduced team cohesion (Gallucci, 2014), though often as a heuristic rather than an empirically validated model.

In organizational contexts, LH has been used to explain motivational withdrawal from performance-related tasks when employees perceive their actions as consistently ineffective. When efforts to contribute—such as through innovation or speaking up—are repeatedly ignored or penalized, individuals may disengage, withhold effort, and avoid taking initiative (Chung et al., 2017; Pinder & Harlos, 2001). These patterns are often accompanied by negative emotional consequences, further reinforcing inaction (Hao et al., 2022). However, and as in other cited domains of application, LH is frequently used here as a metaphorical framework rather than a clearly specified or empirically tested model (Andrieu et al., 2024).

3.3. Civic and societal contexts

LH has been used to understand the psychological consequences of persistent adversity among stigmatized populations such as individuals facing social exclusion, structural discrimination, homelessness, or chronic unemployment. In these contexts, repeated failure to change one's circumstances—often in the face of systemic barriers—has been linked to diminished motivation, reduced agency, and disengagement from support systems (Fisher & Hotchkiss, 2008; Gomez et al., 2015; Rodríguez, 1997).

Beyond individual hardship, the LH framework has also been used to interpret disengagement in response to large-scale crises. When individuals perceive their actions as ineffective against global environmental threats such as climate change, this may give rise to passivity and anxiety mirroring LH symptoms (Landry et al., 2018). Structural constraints can further entrench a sense of collective LH, inhibiting both individual and group-level action (Gunderson, 2023).

Under conditions of structural injustice, researchers have noted that political disempowerment may similarly foster affective states resembling LH—marked by resignation, apathy, and reduced mobilization (Couto & Pilati, 2024; Prilleltensky & Gonick, 1996). While such applications (e.g., environmental concerns, collective action, politics) stretch the theory beyond its original scope, they underscore its heuristic value for understanding how perceived inefficacy can suppress political voice and civic engagement. Yet, the extent to which these responses reflect the core mechanisms of LH (particularly the learned non-contingency between actions and outcomes, and the interpretation of uncontrollability) remains an open question. Originally developed to account for individual consequences, the theory's extrapolation to societal and collective dynamics is largely metaphorical. It also remains unclear whether helplessness can be acquired vicariously—through observing others' repeated failures—rather than through one's direct experience with repeated non-contingency.

4. General Discussion

The accumulated research reviewed here illustrates how the LH theory remains both a theoretically significant and empirically productive construct in psychology. Over the past three decades, its scope has expanded beyond its original behavioral paradigm to encompass cognitive, metacognitive, and neurobiological dimensions, while its applications have extended into diverse research domains in psychology. However, theoretical tensions and methodological challenges have persisted throughout this period.

4.1. The relevance of learned helplessness nowadays

Despite some refinement since its inception and reformulations (1960s-1990s), the LH theory continues to serve as a valuable conceptual lens for understanding patterns of passivity, motivational decline, and impaired problem-solving in the face of perceived uncontrollability. Its enduring relevance stems in part from its capacity to accommodate multiple levels of analysis (from neural circuit dysfunction to cognitive appraisals and contextual moderators), making it somewhat adaptable to a broad range of empirical and applied domains.

Empirically, LH remains useful in contexts where the perception of non-contingency between action and outcome is central to behavioral change (e.g., chronic illness, persistent academic failure, or long-term unemployment). Moreover, recent neuroscientific findings lend biological plausibility to the model, reinforcing its status as more than a purely cognitive or attributional process.

However, the very versatility of the construct, which has kept it relevant, also presents important conceptual and methodological risks. The application of LH across diverse contexts has sometimes resulted in inconsistent operational definitions and blurred conceptual boundaries, with the term used to describe phenomena ranging from specific motivational deficits to broad patterns of disengagement.

4.2. Are we still talking about learned helplessness?

The large body of research conducted under the banner of "learned helplessness" over the past six decades raises a fundamental question: is the research referring to the same construct across studies, populations, and levels of analysis?

One formulation by Peterson et al. (1993) explicitly defines learned helplessness by means of three interrelated components: (1) contingency (i.e., a history of uncontrollability; the objective relation between actions and outcomes; *necessary antecedents*), (2) cognition (i.e., the perception, interpretation, and extrapolation of these contingencies; *mediating processes*), and (3) behavior (i.e., observable patterns of passivity or performance decrements and emotional breakdowns; *observable consequences*). This tripartite model was intended to delimit the construct and provide clear criteria for identifying it across different contexts.

Yet, as the literature expanded, these boundaries have often been blurred. In some cases, the construct is reduced to a single dimension, such as perceived loss of control self-reported on a questionnaire. While perceived uncontrollability is a central feature of LH, it is only one part of a broader phenomenon. With such a narrow operationalization, it becomes questionable whether the phenomenon considered should be labeled "learned helplessness" at all (for an example of a list see Andrieu et al., 2024).

Boundaries between disciplines further hinder the development of coherent conceptualization. Neuroscientific studies tend to equate LH with specific neural or physiological markers of uncontrollability; educational or clinical research often relies almost entirely on attributional style measures; organizational psychology may focus on motivational self-reports. Although each approach yields valuable insights, the risk is that "learned helplessness" becomes a loose label applied to heterogeneous phenomena that share only partial resemblance.

The disciplinary divide runs deeper still: despite LH's behavioral roots, the study of LH in humans has remained overwhelmingly cognitive. The behavior-analytic tradition has investigated non-contingency in humans extensively, but largely under the heading of non-contingent reinforcement in applied settings (e.g., the suppression of self-injury or aggression in clinical populations), rather than as the human state termed "learned helplessness." Two reasons explain this divide. First, LH was defined as a cognitive theory from the outset, positing a subjective representation of action-outcome contingency as

the mediator between objective contingencies and their behavioral effects (Peterson et al., 1993). Second, early behaviorist alternatives that dispensed with such mediation were proposed and tested but were set aside by proponents of the theory in favor of the cognitive account (Maier & Seligman, 2016). As a result, the two traditions have studied closely related operations while rarely engaging one another.

Further complications arise when integrating multiple levels of analysis. While theoretical advances have enriched the construct by incorporating cognitive fatigue, goal regulation, and neurocircuitry, the field has yet to develop robust frameworks that link these levels coherently. The absence of such integration makes it unclear whether findings at the neural level, the cognitive level, and the behavioral level are describing converging expressions of one process, or distinct processes that happen to share superficial similarities.

In this light, we propose that a return to the original tripartite definition may offer a valuable heuristic for future research (Table 1). By exhaustively and jointly assessing contingencies, cognitions, and behaviors, researchers could more clearly delineate the situations in which they are observing LH in the strict sense, as opposed to related but distinct constructs such as mere lack of perceived control, amotivation, or adaptive disengagement. This approach could also improve cross-study comparability and help to determine whether LH can be generalized as a unified phenomenon across contexts, or whether its apparent breadth reflects a mere resemblance among disparate phenomena.

In short, the question is not only whether LH remains relevant, but whether scholars still study it as a coherent construct. Without conceptual discipline and methodological alignment, the term is currently at risk of drifting into a catch-all category for diverse manifestations of passivity and control loss, thereby diluting both its theoretical precision and its explanatory power.

5. Methodological challenges

Given the definitional issues discussed above, it is not surprising that methodological variability has been one of the most persistent obstacles to the coherent advancement of LH theory. At the most basic level, differences in operationalization ranging from multi-component assessments to single-item measures or qualitative appraisal of perceived control make cross-study comparison difficult.

A second challenge lies in research design. The original paradigm relied on controlled manipulations of action–outcome contingencies; many contemporary studies instead infer uncontrollability indirectly or embed it in complex, real-world contexts where multiple causal factors are at play. While these settings strengthen ecological validity, they make it harder to isolate uncontrollability as the causal mechanism. Measurement timing also remains problematic. Most LH studies measure outcomes shortly after exposure to uncontrollable conditions, yet longitudinal evidence of the persistence and generalization of helplessness is relatively scarce. The lack of temporal depth limits our understanding of whether observed effects reflect transient states or stable changes in motivation, cognition, and behavior.

Furthermore, cross-level integration is rarely attempted. Neural markers, cognitive appraisals, and behavioral indicators are typically studied in isolation, making it difficult to establish whether findings from one level of analysis correspond to, or diverge from, those at another.

Addressing these methodological challenges will require not only more rigorous operationalization and design, but also a deliberate effort to bridge levels of analysis and disciplinary perspectives.

6. Limitations

A rigorous framework should be held to its own standards, and ours is no exception. We therefore submit the tripartite definition of LH to one of its most fundamental challenges: the behavior-analytic objection developed earlier in the manuscript. If the non-contingent operation alone is sufficient to produce the deficits, then our middle component, cognition, is a contestable term: by requiring it, the tripartite definition risks building the cognitive assumption into the definition of the phenomenon, prejudging a question that the data on non-contingent operations leave open. We take this objection seriously, but we read it as a reason to constrain the cognitive component rather than to abandon it. The mediating term earns its place only when it is independently measured and does explanatory work beyond redescribing the behavior, most clearly where a purely functional account is silent: individual differences (the same schedule yields helplessness in some and resilience in others), transfer across situations, and the link to clinical outcomes.

The tripartite model is also not without limitations of a more practical kind, and may not capture the full range of dynamics observed in contemporary applications of LH, especially in complex real-world settings. The clear separation into three components is analytically useful, yet in practice these processes are often intertwined, making it challenging to isolate them. Moreover, an overly rigid and ecologically weak application of the criteria could introduce the risk of overlooking cases where partial patterns of uncontrollability still produce significant motivational and affective consequences.

Nevertheless, these caveats do not diminish the value of the framework; rather, they point to the need for a flexible but structured approach, one that uses the three components as an organizing scaffold while allowing adaptation to the unique characteristics of each research domain. In this spirit, the most informative future programs may be those that hold the contingency manipulation constant and ask what, if anything, the cognitive and neural levels add to prediction, rather than assuming mediation from the outset.

7. Conclusions and future directions

More than half a century after its introduction, LH remains both a

Table 1
Levels of operationalization of learned helplessness, developed from Peterson et al. (1993).

Operationalization	Learned Helplessness Components			Conclusions
	Contingency Assessment	Mediating Cognitions	Behavioral Assessment	
Full	Objective measurement of the repeated relationship between actions and outcomes (e.g., controllable vs. uncontrollable tasks)	Measures of perceived control[a] AND its interpretation[b] AND cognitive fatigue[c]	Observation/recording of passivity vs. activity or performance deficits in relevant tasks	Ensures construct validity across levels of analysis
Partial	Indirect or inferred measure (e.g., manipulation assumed to create uncontrollability without direct check)	Measures of perceived control[a] OR its interpretation[b] OR cognitive fatigue[c]	Behavior/performance inferred from self-reported or proxy indicators	May miss components of the LH process
Questionable	Not assessed; assumed based on context or entirely absent	Self-reported control loss only, no attributional or interpretative measures	No behavioral component; purely self-reported	Likely measuring a related but distinct phenomenon

Note. [a]Degree of learned dependance between actions and outcomes. [b]Depending on the theoretical lens, can relate to attributions (Abramson et al., 1978), goal evaluations/similarity/importance (Boddez et al., 2022; Mikulincer & Lifshin, 2022). [c]Relates to the informational reformulation of LH and indicates the degree to which people try to find alternative hypotheses to resolve the tension of non-contingency.

popular and contested construct. Its conceptual resilience lies in its ability to integrate behavioral, cognitive, and neurobiological processes under a single explanatory framework. Yet, as we have argued, this versatility comes at a cost: definitional drift, inconsistent operationalization, and disciplinary fragmentation that threaten not only its theoretical coherence but also its capacity to offer precise and differentiated analyses of the phenomena to which it is applied.

Future progress will depend on restoring conceptual discipline without sacrificing the model's capacity to adapt to diverse contexts. The tripartite definition offered by Peterson et al. (1993) provides a valuable framework for this task, serving both as a heuristic for study design and as a benchmark for evaluating whether "learned helplessness" is being investigated in its intended sense.

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Clement F.A. Andrieu: Conceptualization, Investigation, Methodology, Project administration, Validation, Visualization, Writing – original draft, Writing – review & editing. **Isabelle Milhabet:** Conceptualization, Investigation, Methodology, Resources, Writing – original draft, Writing – review & editing.

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